

Our Ref: BGM/CS/ [MED-ID]

[DATE]

Clinic Letter [DATE]

[PROVIDER-NAME]

[ADDRESS]

[ADDRESS]

[ADDRESS]

[ADDRESS]

[ADDRESS]

[POSTCODE]

Division B

Care Group - Emergency & Specialist Medicine Respiratory Medicine

Minerva House C-Level, Mailpoint [ADDRESS]

[ADDRESS]

[POSTCODE]

Telephone: [PHONE]

Dear [PROVIDER-NAME] [NAME] , [ADDRESS]. [POSTCODE]

DOB: [DATE] , Hospital Number: [MED-ID] , NHS Number: [NHS-NO]

Diagnoses:

Pulmonary sarcoidosis

Crohn's disease (small bowel resection) currently quiescent

Exercise programme

I was pleased to see Malcolm with his wife today. He looks well and is currently engaged in an exercise programme with resulting weight loss down to 98.6kgs today. His job is physical and struggling with this either with breathlessness or fatigue. His last lower respiratory tract was in September last year which took a while to settle. Apart from occasional wheeze, he is breathless.

His current treatment is Azathioprine 25 mgs once a day, Prednisone 5 mgs once a day, Allopurinol once a day, Simvastatin, Alendronic acid 70 mgs per week.

His last chest x-ray showed improvement in the right hilar change with clear lung fields.

I have been reassuring to him. His lung function tests remain stable with a mild obstructive consistent with endobronchial sarcoid.

I have not offered any extra treatment and I am happy with the treatment he is on which is appropriate for his Crohn's.

I will keep him under review at six monthly intervals and I have booked him for lung function test, chest x-ray as well as bloods in October.

Yours sincerely

[NAME]

[NAME]

[DICTATION-META]

[PROVIDER-NAME]

Consultant Respiratory Physician and Honorary Associate Professor in Medicine